

SKIN LUMP/S

Refer same week the patient with a mole that:

- Is irregular in shape or colour
- Changed in size, shape or colour
- Differs from surrounding moles
- Is > 6mm wide
- Bleeds easily
- Itches

If painful, firm, red, warm lump which softens in the centre to discharge pus, **boil/abscess** likely → 68.

Round, raised papules with rough surfaces



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Warts likely

- Usually on hands, knees or elbows but can occur anywhere.
- Plantar warts on the soles of the feet are thick and hard with black dot/s.
- Reassure that warts often resolve spontaneously.
- If treatment desired:
 - Soften wart/s by soaking in warm water for 5 minutes at night and scrub gently with clean nail file.
 - After drying well, apply **petroleum jelly** to surrounding skin to protect it, then apply **salicylic acid 15-30%** to wart, and cover with plaster.
 - Repeat every night and continue for a week after wart has come off.
- If extensive warts or plantar wart in diabetic, refer.

Small, skin-coloured pearly bumps with central dimples



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Molluscum contagiosum likely

- Test for HIV → 110.
- Reassure that lesions often resolve spontaneously after several years or with ART.
- If treatment desired: open molluscum with sterile needle and apply **tincture of iodine BP** to center of each lesion.
- Refer if:
 - Extensive
 - Lesions on eyelid
 - Intolerable and not responding to treatment

Painless, purple/brown lumps on skin



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Kaposi's sarcoma likely

- Lesions vary from isolated lumps to large ulcerating tumours.
- May also appear in mouth and on genitals.
- Test for HIV → 110.
- Refer for biopsy to confirm diagnosis and for further management.

Smooth, well defined lump beneath skin

Round, firm lump. May have central hole and discharge white substance.



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Epidermoid cyst likely
Usually found on face and trunk, uncommon on limbs.

- If not infected, reassure there is no need to treat.
- If infected (skin red, warm, painful):
 - If fluctuant, arrange incision and drainage. If on face, refer instead.
 - Give **flucloxacillin** 500mg 6 hourly or **cefalexin** 500mg 6 hourly for 5 days.
 - If severe penicillin allergy¹, give instead **azithromycin** 500mg daily for 3 days.
- If intolerable or recurrent infections, arrange for excision once infection resolved.

Soft, doughy lump which is painless and moves easily.



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Lipoma likely
Usually found on trunk or upper limb.

- Reassure lump will not become cancer and usually does not need removal.
- Refer if:
 - > 3cm
 - Causing pain or discomfort
 - Getting bigger
 - Firm or deep beneath skin
 - New lump that persists > 1 month
 - Intolerable

Red papules, pustules, nodules and blackheads, usually on face.
May involve chest, back and upper arms



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Acne likely

- Advise to wash skin with mild soap twice a day and to avoid picking, squeezing and scratching.
- Advise to avoid oily cosmetics and hair products.
- If blackheads only:
 - Apply **tretinoin 0.05%** cream sparingly at night until better, for at least 6 weeks. Avoid if pregnant or breastfeeding and limit sun exposure. Acne may worsen before improving. Review after 6 weeks.
- If red and swollen areas:
 - Apply instead **benzoyl peroxide 5%** gel to affected areas in morning. Wash off in evening. If no better and tolerating gel, apply twice daily and give **doxycycline**² 100mg daily with meals, for 3 months.
- If woman needing contraception, advise combined oral contraceptive → 154.
- Advise that response may take several weeks to months.
- If severe or no response after 6 months of treatment, refer.

If diagnosis uncertain, refer.

¹History of angioedema, anaphylaxis or urticaria. ²Doxycycline may interfere with oral contraceptive, advise patient to use condoms as well. Avoid if pregnant or breastfeeding. Avoid giving together with iron or antacids.